



## Guideline

# Death in Hospital

## Scope

| Site     | Service/Department/Unit | Disciplines |
|----------|-------------------------|-------------|
| SCGOPHCG | Hospital-Wide           | All Staff   |

## Introduction

This guideline applies to all patient deaths that occur within the Sir Charles Gairdner Osborne Park Health Care Group (SCGOPHCG), including those under the care of Homelink and Rehabilitation in the Home (RITH) services and those who decess whilst on leave.

Customs dictated by the religious beliefs or ethnicity of the deceased will be respected and accommodated to the extent possible, but within law.

## Risk Statement

Non-compliance with this policy will:

|                                       |                                     |                                   |                                     |
|---------------------------------------|-------------------------------------|-----------------------------------|-------------------------------------|
| Breach legislative requirements       | <input checked="" type="checkbox"/> | Impact on Patient Quality of Care | <input checked="" type="checkbox"/> |
| Breach National/State/Hospital Policy | <input checked="" type="checkbox"/> | Impact on Patient Safety          | <input type="checkbox"/>            |
| Breach professional standards         | <input checked="" type="checkbox"/> | Misconduct                        | <input type="checkbox"/>            |
| Breach SCGOPHCG Mission & Values      | <input type="checkbox"/>            | Staff Safety                      | <input type="checkbox"/>            |
| Other:                                | <input type="checkbox"/>            |                                   |                                     |

## Guideline Principles

This guideline aims to assist staff in the process of certification of death, the identification of coronial and non-coronial cases and reporting obligations.

This guideline should be read in conjunction with the [WA Health Review of Death Policy](#), [NMHS Morbidity and Mortality Policy](#), [NMHS Release of the Deceased Policy](#), [Last Offices Nursing Practice Guideline](#) and the [SCGOPHCG Bereavement Guideline](#).

## Procedure

### 1. Certification and Notification of Death

Certification of life extinct is done by the team doctor or after hours by the on-call doctor. A record of life extinct is made in the medical record.

All deaths, other than those reportable to the coroner, must have a Medical Certificate of Cause of Death (BDM202), or a Medical Certificate of Cause of Stillbirth or Neonatal Death (BDM 201) completed. These are referred to as the Death Certificate and are available in hard copy only from the ward clerk or from the Stillbirth pack in Birth Suite.

In all instances the Death in Hospital (DIH) form MR 604/37.1 must be completed. This is to be done no later than 48 hours after death, by the medical officer responsible for the care of the patient at the time of death. All sections of the form must be completed, and

the original filed in the medical record.

This form clearly outlines the following requirements:

- deaths reportable to the Coroner,
- how to report the death to the Coroner,
- notification of reportable deaths under The Coroners Act 1996,
- other reporting obligations
- additional hospital requirements

As soon as possible notify the following:

- Ward or After-Hours Senior Registered Nurse
- The patient's Next of Kin (NOK)
- The on-call Consultant
- The treating Consultant
- Patient's GP
- Consideration of notifying Pastoral Care

## 2. Care of the Deceased, non-Coronial

Also refer to the [Last Offices Nursing Practice Guideline](#)

**Sir Charles Gairdner Hospital (SCGH):** After the Last Offices the deceased may be transferred directly to the State Mortuary

**Osborne Park Hospital (OPH):** After the Last Offices deceased patients are removed directly from the ward by the family nominated funeral director. If this is not possible the Holding Room is available for up to 24 hours, prior to transfer to a mortuary or a funeral director. Family and/or next of kin (NOK) are given the opportunity (where practicable), to view their deceased relative on the ward prior to transfer to the holding room. Refer to the [OPH Release of the Deceased Patient from Holding Room to Funeral Home](#) guideline.

The following documentation is to be transferred, with the patient to the mortuary/holding room:

- Admission form stating details of Next of Kin
- Medical Certificate of Cause of Death (BDM 202/201)
- Certificate of Medical Attendant (Form 7 WA Cremation Act)
- Death in Hospital form MR 604/37.1

Families who wish to arrange for burial of a relative without the intervention of a funeral director need to be aware of the potential problems associated with transfer of a body from the hospital, subsequent storage, and burial. The family should be referred to a funeral director for advice.

### 2.1 Body Donation

In some instances, a person may be a registered donor with a Body Donation Program. In Western Australia, the licensed Body Donation Program is run by the University of Western Australia (UWA).

The next of kin of the registered donor will be responsible for notifying the Body Donation Program Coordinator upon the person's death. The next of kin can also request that the contact made by medical staff or funeral director on their behalf.

The Body Donation Program Coordinator then contacts the medical staff, certifying the death, to ascertain suitability of the donor for the bequest program. If the donor is

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accepted, UWA will take care of the transfer of the donor to the University and notify and register the death with the relevant government authorities.

Refer to [UWA Body Donation Program](#) for more details.

## 2.2 Deceased Patient with no Family/Next of Kin and/or Ward of the state.

Deceased patients with no family or Next of Kin (NOK) are placed under the care of the Public Trustee. If the deceased patient is already under the care of the Public Trustee at the time of death, the senior nurse contacts the office of the Public Trustee (Phone: 1300 746 116) to advise of death. Senior nurse also informs PathWest Mortuary (Phone: 63834884) of the death.

State Mortuary staff commences the process of establishing the existence of any NOK or distant relative. Once confirmed there is no person able or willing to take responsibility for the body, the deceased becomes an Unclaimed Body at which time Mortuary staff will complete and forward the Intestate form to the Public Trustee Office where legislative and state procedures are followed.

Where the deceased person is not under the care of the Public Trustee but has no family or NOK:

- The senior nurse contacts the Social Worker involved in the patient's care to notify them of the patient's death.
- Senior nurse contacts PathWest Mortuary (63834884) to advise of death.
- OPH will contact CINTRA (Business hours mobile: 0418981706 and A/H 0400901771 who will transport the deceased person to the State Mortuary.
- OPH will be responsible for the transport cost of the deceased person from OPH Holding Room to the State Mortuary.
- SCGH will transport deceased person direct to mortuary.
- Ensure patient identification labels remain on deceased person.

## 3. Care of the Deceased - Coronial Cases

In accordance with the *Coroners Act 1996*, if a Post-Mortem Examination (PME) is carried out to determine the cause of death, it must be carried out by a Coroner's pathologist at the direction of a Coroner. This is known as a **Coronial PME**.

Refer to Section 5 of the [Death in Hospital Form](#) for reportable death criteria.

- Reportable deaths must be reported to the coroner immediately.
- A photocopy of the medical record is to be made prior to release of the original to the coroner's office.

Unless coronial staff indicates otherwise:

- All medical invasive devices e.g. IV lines including the attached pump, NG tubes, endotracheal tube (ETT), must be left in-situ and secured to the body;
- All medication, ampoules, syringes, containers, etc. which had or still do contain substances administered to the patient are preserved and kept with the body, unless they have been placed in a sharps container also containing items from other patients;
- Any injury or wound is NOT to be cleaned or interfered with in any way after death;
- Any tissue, foreign material, skin, etc. (including blood samples) removed during treatment must be made available for forensic examination.
- All clothing associated with the patient is to be retained and either left on the body or if already removed, sealed in a paper bag and kept with the patient.

## Coronial Body Bagging and Transportation Process (Cintra, Police & PathWest State Mortuary).

**OPH/Woods Ward:** Contact the Western Australian (W.A.) Police Coronial Investigation Squad (CIS) on 08 9267 5700 (24 hours-a-day, 7 days-a-week). CIS will coordinate for CINTRA to facilitate bags, tags and P98 and the transportation of the deceased person to the state mortuary.

**SCGH:** Contact the WA Police Coronial Investigation Squad (CIS) on 08) 9267 5700 (24 hours-a-day, 7 days-a-week). The deceased will be transported directly to the mortuary on site not requiring CINTRA.

If there are any issues with the above process, contact State Mortuary PathWest or after hours through PathWest Security (via switch \*91).

## 4. Non-Coronial Post-mortem Examinations

In accordance with the *Human Tissue and Transplant Act 1982*, a **non-coronial post-mortem examination (PME)** may be carried out by a pathologist to determine the extent of disease, or for the purpose of teaching, research and quality improvement, but not to determine the cause of death.

Once a Medical Certificate of Cause of Death has been completed, a non-coronial PME may be considered. Medical Staff should contact the on-call Post-Mortem Coordinator for further information. Refer to the [Non-Coronial Post-Mortem Examinations](#) page on HealthPoint

## 5. Other Reporting Obligations

The DIH form also sets the following reporting requirements

- Under the *Health (Miscellaneous Provisions) Act 1911*, the following deaths must be reported to the Chief Health Officer:
  - A maternal death (arising from pregnancy or childbirth or associated with complications)
  - A death involving a child who is stillborn (> than 20 weeks gestation), or under the age of 1 year
  - A death that occurred within 48 hours of administration of anaesthetic or as a result of complications arising from the same.
- The Chief Psychiatrist must be informed of any death of a mental health patient:
  - While under the care of any mental health or other health service
  - Within 28 days of discharge or deactivation from mental health services.
- Severity Assessment Code (SAC) 1 Clinical Incidents
 

Medical staff are to consider if a clinical incident has occurred and if so report this in the Datix CIMS database as per [NMHS Clinical Incident Management Policy](#). See the [SCGOPHCG Clinical Risk Management HealthPoint page](#) for more information.

  - Reporting to the DoH is conducted by SCGOPHCG Clinical Risk Management team.

In accordance with the [MP0098/18 Review of Death Policy](#) and the [NMHS Morbidity and Mortality policy](#), all deaths that occur in public hospitals and licensed private health care facilities in Western Australia are required to be assessed and reviewed in a timely manner (within four (4) months of the date of death).

## Related Documents

- [Health \(Miscellaneous Provisions\) Act 1911](#)
- [Coroners Act 1996](#)
- [Human and Tissue Transplant Act 1982](#)
- [Mental Health Act 2014](#)
- [MP0098/18 Review of Death Policy](#)
- [NMHS Morbidity and Mortality Review Policy](#)
- [SCGOPHCG Bereavement Policy](#)
- [SCGOPHCG Organ and Tissue Donation after Death Policy](#)

## State Mortuary and Bereavement Centre

State Mortuary/Bereavement Viewing Facility [mortuary.pathwest@health.wa.gov.au](mailto:mortuary.pathwest@health.wa.gov.au)

T: 63834895, After Hours: 64572536

[Office of the State Coroner](#)

## Authorisation & Version Control

Authorisation & Version Control

|                                                                                            |                                                                                                                                                                         |                      |            |                          |            |
|--------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------|------------|--------------------------|------------|
| Policy Sponsor:                                                                            | SCGOPHCG Area Director Clinical Services                                                                                                                                |                      |            |                          |            |
| Policy Contact:                                                                            | SCGOPHCG Manager Clinical Risk                                                                                                                                          |                      |            |                          |            |
| Version #:                                                                                 | 2                                                                                                                                                                       | Last Reviewed:       | 19/05/2023 | Review Due:              | 19/05/2026 |
| Reference #                                                                                | HG194                                                                                                                                                                   | Consumer Engagement: | N/A        | Aboriginal Health ISD #: | N/A        |
| Approved by:                                                                               | SCGOPHCG Area Director Clinical Services                                                                                                                                |                      |            | Date:                    | 13/06/2023 |
| Endorsed by:                                                                               | SCGOPHCG Policy and Guideline Committee                                                                                                                                 |                      |            | Date:                    | 13/06/2023 |
| Applicable Standards:                                                                      |   |                      |            |                          |            |
| Printed or personally saved electronic copies of this document are considered uncontrolled |                                                                                                                                                                         |                      |            |                          |            |

This document can be made available in alternative formats on request for a person with a disability.

## Appendix 1: Death in Hospital (DIH) Form

sample only - do not print

USE PATIENT LABEL WHEN AVAILABLE

|                                                                                                                                                          |                                                                                                                                                                                                                                                                                                 |
|----------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <p>SIR CHARLES GAIRDNER OSBORNE PARK<br/>HEALTH CARE GROUP</p> <p style="text-align: center; font-weight: bold; font-size: 1.2em;">DEATH IN HOSPITAL</p> | <p>URN: <span style="border: 1px solid black; display: inline-block; width: 100px; height: 20px;"></span></p> <p>Family name: _____</p> <p>Given name(s): _____</p> <p>Gender: _____ DOB: <span style="border: 1px solid black; display: inline-block; width: 100px; height: 20px;"></span></p> |
|----------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|

SECTION 1: EXTINCTION OF LIFE

**Note:** Certification of Life Extinct is to be documented in the medical record by the doctor who conducts the examination of the body.

Death occurred in: \_\_\_\_\_ ED ☐ Ward ☐ Theatre ☐ Other ☐ While on leave from hospital ☐

Doctor Certifying Life Extinct: Name: \_\_\_\_\_ Position: \_\_\_\_\_

Signature: \_\_\_\_\_ Date: \_\_\_\_\_

Date of Death: \_\_\_\_/\_\_\_\_/\_\_\_\_ dd/mm/yy Time of Death: \_\_\_\_:\_\_\_\_ 00:00 hours

SECTION 2: IS THE DEATH REPORTABLE TO THE CORONER?

1. Is the cause of death unknown or uncertified by a medical practitioner? \_\_\_\_\_ ☐ Yes ☐ No
2. Has the death or does the death appear to be have occurred in suspicious circumstances?  
e.g. Has the death possibly resulted from a criminal act? \_\_\_\_\_ ☐ Yes ☐ No
3. Was the death or does the death appear to have been unexpected or unnatural?  
e.g. Complication following administration of a medication, diagnostic, medical or surgical procedure \_\_\_\_\_ ☐ Yes ☐ No
4. Has the death or does the death appear to have occurred, in or following violent circumstances?  
e.g. Physical or sexual assault, domestic dispute \_\_\_\_\_ ☐ Yes ☐ No
5. Has the death or does the death appear to have resulted, directly or indirectly from injury?  
e.g. Fall, motor vehicle, self harm \_\_\_\_\_ ☐ Yes ☐ No
6. Has the death occurred during anaesthesia? e.g. General anaesthesia \_\_\_\_\_ ☐ Yes ☐ No
7. Did the death possibly occur as a result of, or does it appear to have resulted from, anaesthesia? \_\_\_\_\_ ☐ Yes ☐ No
8. Immediately prior to the death was the deceased a person:
  - Under the control, care or custody of the WA Police Force, Prison Service or Child Protection and Family Support \_\_\_\_\_ ☐ Yes ☐ No
  - Admitted to a centre under the *Alcohol and Other Drugs Act 1974* \_\_\_\_\_ ☐ Yes ☐ No
  - An involuntary patient, apprehended, detained or absent without leave under the *Mental Health Act 2014* \_\_\_\_\_ ☐ Yes ☐ No
9. Is the deceased person's identity unknown? \_\_\_\_\_ ☐ Yes ☐ No
10. To your knowledge has any one expressed any concerns regarding the cause of the deceased person's death or medical treatment? \_\_\_\_\_ ☐ Yes ☐ No

If you have answered **YES** to any of the above questions, the death is **REPORTABLE** to the Coroner, and the most recent medical records leading up to the person's death must be provided to the police immediately or, in any case, to the State Mortuary not more than 24 hours following death. This is to ensure that the post mortem examination can be conducted in a timely manner.

Where records are paper-based, the originals are to be provided with copies retained by the health service for any ongoing purposes. Should further medical information be requested by the police/coroner these records should then be provided as soon as practicable.

SECTION 3: HOW TO REPORT A DEATH TO THE CORONER

To report a death to the Coroner, or to seek guidance about reportable deaths:

**METROPOLITAN PERTH:**  
Contact the **WA Police Coronial Investigation Squad (CIS)** on 08 9267 5700 (24 hours-a-day, 7 days-a-week). Scan and email this form to: [CoronialInvestigationSquad@police.wa.gov.au](mailto:CoronialInvestigationSquad@police.wa.gov.au)

**COUNTRY WA SETTINGS:** Contact the **local police**. If further guidance or assistance is required, contact the CIS on 08 9267 5700. The Office of the State Coroner (OSC) can be contacted on 08 9425 2900 (business hours) or on 0419 904 478 (after 4:30pm and on weekends) if the CIS and local Police are unable to assist. **See over**

 DEATH IN HOSPITAL  
SCGH MR604 / OPH MR37.1

 HOHGA/MPR604A  
M077  
08/23

Sample only do not print

USE PATIENT LABEL WHEN AVAILABLE

|                                                                                                                                                             |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |
|-------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| SIR CHARLES GAIRDNER OSBORNE PARK<br>HEALTH CARE GROUP<br><br><div style="text-align: center; font-weight: bold; font-size: 1.2em;">DEATH IN HOSPITAL</div> | <div style="text-align: right;">URN: <span style="border: 1px solid black; display: inline-block; width: 100px; height: 20px;"></span></div> <div style="margin-top: 10px;">Family name: _____</div> <div style="margin-top: 10px;">Given name(s): _____</div> <div style="display: flex; justify-content: space-between; margin-top: 10px;"> <div>Gender: _____</div> <div>DOB: <span style="border: 1px solid black; display: inline-block; width: 100px; height: 20px;"></span></div> </div> |
|-------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|

**SECTION 4: NOTIFICATION OF REPORTABLE DEATHS UNDER THE CORONERS ACT 1996**

On notification you will be asked for information on the circumstances surrounding the death, which should be conveyed to the coronial delegate (e.g. the CIS / police officer to whom the death is reported). You should also consider the answer to the question **"Was death an inevitable consequence of the deceased person's primary illness or condition regardless of appropriate resuscitation, anaesthesia, or surgery?"** In cases when the answer is 'yes' a post mortem may not be necessary.

CIS / police officer notified by phone: Date: \_\_\_\_/\_\_\_\_/\_\_\_\_ dd/mm/yy Time: \_\_\_\_ : \_\_\_\_ 00:00 hours

Name of CIS / police officer: (please print) \_\_\_\_\_

Name of Doctor reporting: (please print) \_\_\_\_\_

Consultant notified: Yes ☐ No ☐ Contact Number: \_\_\_\_\_ Sign: \_\_\_\_\_

If the death is reportable a copy of this Form should also be placed in the deceased person's Hospital Medical Record as well as the Hospital's Coronal Investigation File.

Where the death is NOT reportable:

- the original copy of this Form must be filed in the deceased person's Hospital Medical Record
- you may complete the following Forms:

|                                                               |            |                              |                             |
|---------------------------------------------------------------|------------|------------------------------|-----------------------------|
| 1. Medical Certificate Cause of Death (BDM 202/201) and       | Completed? | Yes <input type="checkbox"/> | No <input type="checkbox"/> |
| 2. Certificate of Medical Attendant (Form 7 WA Cremation Act) | Completed? | Yes <input type="checkbox"/> | No <input type="checkbox"/> |

**SECTION 5: OTHER REPORTING OBLIGATIONS**

**5.1 REPORTABLE DEATHS UNDER THE HEALTH (MISCELLANEOUS PROVISIONS) ACT 1911**

Is the death:

- a maternal death (arising from pregnancy or childbirth or associated with complications) \_\_\_\_\_ ☐ Yes ☐ No
- one involving a child who is stillborn (> than 20 weeks gestation), or under the age of 1 year \_\_\_\_\_ ☐ Yes ☐ No
- one that occurred within 48 hours of administration of anaesthetic or as a result of complications arising from the same \_\_\_\_\_ ☐ Yes ☐ No

If you have answered YES to any of the above questions, the death is reportable to the Chief Health Officer. Information on reporting processes is found on the WA Department of Health Statutory medical notifications website: [http://ww2.health.wa.gov.au/Articles/A\\_E/About-statutory-medical-notifications-in-Western-Australia](http://ww2.health.wa.gov.au/Articles/A_E/About-statutory-medical-notifications-in-Western-Australia)

**5.2 DEATHS REPORTABLE TO THE CHIEF PSYCHIATRIST**

The Chief Psychiatrist is to be informed of any death of a mental health patient while under the care of any mental health or other health service, and deaths that occurred within 28 days of discharge or deactivation from mental health services. Is this a reportable death to the Chief Psychiatrist? \_\_\_\_\_ ☐ Yes ☐ No

For further information refer to the Chief Psychiatrist website:  
<https://www.chiefpsychiatrist.wa.gov.au/monitoring-reporting/notifiable-incidents/>

**5.3 SEVERITY ASSESSMENT CODE (SAC) 1 CLINICAL INCIDENTS**

The death may reflect the occurrence of a SAC 1 clinical incident. SAC 1 clinical incidents include all clinical incidents / near misses where serious harm or death is or could be specifically caused by healthcare rather than the patients underlying condition or illness. For further information refer to the Clinical Incident Management (CIM) Policy:  
<http://ww2.health.wa.gov.au/Health-for/Health-professionals/Safety-and-quality>

**SECTION 6: ADDITIONAL HOSPITAL REQUIREMENTS (as determined by local Policies / Guidelines)**

|                                                                                                                            |                              |                             |                               |                              |                             |
|----------------------------------------------------------------------------------------------------------------------------|------------------------------|-----------------------------|-------------------------------|------------------------------|-----------------------------|
| Donor Coordinator Notified                                                                                                 | Yes <input type="checkbox"/> | No <input type="checkbox"/> | Discharge summary completed   | Yes <input type="checkbox"/> | No <input type="checkbox"/> |
| Permission for postmortem                                                                                                  | Yes <input type="checkbox"/> | No <input type="checkbox"/> | Bereavement support           | Yes <input type="checkbox"/> | No <input type="checkbox"/> |
| Nursing home notified                                                                                                      | Yes <input type="checkbox"/> | No <input type="checkbox"/> | General Practitioner notified | Yes <input type="checkbox"/> | No <input type="checkbox"/> |
| Next of kin notified as designated in the Hospital Medical Record <input type="checkbox"/> Yes <input type="checkbox"/> No |                              |                             |                               |                              |                             |